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Apo-Alendronate 70mg Tablets

Alendronate Sodium Trihydrate equivalent to Alendronic Acid 70mg

CLINICAL PHARMACOLOGY

Alendronate is a bisphosphonate that binds to bone hydroxyapatite and specifically inhibits the activity of osteoclasts, the bone-resorbing cells. Alendronate reduces bone resorption and formations are coupled during bone turnover.

Daily oral doses of alendronate 5, 20 and 40 mg for six week, in postmenopausal women produced biochemical changes indicative of dose-dependent inhibition of bone resorption, including decreases in urinary calcium and urinary markers of bone collagen degradation (such as deoxypyridoline and cross-linked N-telopeptides of type 1 collagen). These biochemical changes tended to return toward baseline values as early as 3 weeks following the discontinuation of therapy with alendronate and did not differ from placebo after 7 months.

As a result of inhibition of bone resorption, asymptomatic reduction in serum calcium (approximately 2 %) and phosphate (approximately 4 to 6 %) concentrations were also observed following treatment with alendronate, were evident the first month after the initiation alendronate 10 mg; No further decreases in serum calcium were observed for the five-year duration of treatment; however, serum phosphate returned toward pre-study levels during years three through five.

Pharmacokinetics

Absorption: Relative to an intravenous (IV) reference dose, the bioavailability of alendronate in woman was 0.7 % for doses ranging from 5 to 40 mg when administered two hours before a standardizing breakfast. The bioavailability in men was similar to that in woman (0.6 %). Bioavailability was decreased (by approximately 40 %) when alendronate was administered either 0.5 or 1 hour before a tenderizing breakfast. In studies of osteoporosis, it was effective when administered at least 30 minutes before first food or drink of the day. Bioavailability was negligible whether alendronate was administered with or up to two hours after a standardizing breakfast. Concomitant administration of alendronate with coffee and orange juice reduces bioavailability by approximately 60 %.

Distribution: The mean steady-state volume of distribution, exclusive bone, is at least 28 L in human. Concentrations of drug in plasma following therapeutic oral doses are too low (less than 5 ng/mL) for analytical detection. Protein binding in human plasma is approximately 78 %.

Metabolism: There is no evidence that alendronate is metabolized in animals or human.

Excretion: Mainly eliminated through kidney.

INDICATION AND CLINICAL USE

Apo-Alendronate is indicated for the treatment of osteoporosis in postmenopausal women.

DOSAGE AND ADMINISTRATION

Treatment of osteoporosis in postmenopausal women The recommended dosage is one 70mg tablet once weekly.

No dosage adjustment is necessary for the elderly or for patients with mild-to-moderate renal insufficiency (creatinine clearance 35 to 60 mL/min). Apo-Alendronate is not recommended for patients with

more severe renal insufficiency (creatinine clearance <35mL/min) due to lack of experience.

Mode of Administration

Apo-Alendronate (alendronate sodium) must be taken at least one-half hour before the first food, beverage, or medication of the day with plain water only. Other

beverages (including mineral water), food, and some medications are known to reduce the absorption of Apo-Alendronate (see Drug Interactions).

Waiting less than

30 minutes will lessen the effect of Apo-Alendronate by decreasing its absorption into the body.

Apo-Alendronate should only be taken upon arising for the day. To facilitate delivery to the stomach and thus reduce the potential for esophageal irritation, an Apo-Alendronate tablet should be swallowed with a full glass of water (200-250mL). Patients should not lie down for at least 30 minutes and until after their first food of the day.

Apo-Alendronate should not be taken at bedtime or before arising of the day. Failure to follow these instructions may increase the risk of esophageal adverse experiences (see Warnings and Precautions and Dosage and Administration).

All patients must receive supplemental calcium and Vitamin D, if delivery intake is inadequate.

CONTRAINDICATIONS

Hypersensitivity to any component of this product; abnormalities of the esophagus which delay esophageal emptying such as stricture or achalasia, inability to stand or sit upright for at least 30 minutes; hypocalcemia.

WARNINGS / PRECAUTIONS

Apo-Alendronate, like other bisphosphonates, may cause local irritation of the upper gastrointestinal mucosa.

Esophageal adverse experiences, such as esophagitis, esophageal ulcers and esophageal erosions, and rarely esophageal structure, have been reported in patients receiving treatment with bisphosphonates. In some cases these have been severe and required hospitalization.

The risk of severe esophageal adverse experiences appears to be greater in patients who lie down after taking Apo-Alendronate and/or who fail to swallow it with a full glass of water, and/or who continue to take it after developing symptoms suggestive of esophageal irritation. Because of possible irritant effects of Apo-Alendronate on the upper gastrointestinal mucosa, caution should be used when it is given to patients with active upper gastrointestinal problems, such as dysphagia, esophageal diseases, gastritis, duodenitis or ulcer.

To facility deliver to the stomach and thus reduce the potential for esophageal irritation patients should not chew or suck on the tablet because of a potential for oropharyngeal ulceration; patients should be instructed to swallow Apo-Alendronate with a full glass of water and not to lie down for least 30 minutes and until after their first food of the day.

Material Code: 406882	ECL Common Text#: N/A	Description: INS PHM ALENDRONATE Na TAB
Material Code REF: N/A		

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160 mm

35 mm

280 mm

Patients should be specifically instructed no to take Apo-Alendronate at bedtime or before arising for the day; patients should be informed that failure to follow these instructions may increase risk of esophageal problems.

Apo-Alendronate is not recommended for patients with creatinina clearance < 35 mL/min. Before initiating treatment with Apo-Alendronate the hypocalcemia and deficiency of vitamin D should be ascertained.

Patients treated with glucocorticoids should receive adequate amounts of calcium and vitamin D.

Localized osteonecrosis of the jaw (ONJ), generally associated with tooth extraction and/or local infection with delayed healing, have been reported rarely with oral bisphosphonates. Most reported cases of bisphosphonate-associated ONJ have been in cancer patients treated with intravenous bisphosphonates. Patients who develop ONJ should receive appropriate care by an oral surgeon. Bone, joint, and/or muscle pain has been reported in patients taking bisphosphonates. These symptoms have rarely been severe and/or incapacitating. The time to onset of symptoms varied from one day to several months after starting treatment. Most patients had relief of symptoms after stopping treatment. Patients should be instructed that if they miss a dose of APO-ALENDRONATE once weekly, they should take one tablet on the morning after they remember. They should not take two tablets on the same day but should return to taking one tablet once a week, as originally scheduled on their chosen day. Due to the positive effects of APO-ALENDRONATE in increasing bone mineral, small, asymptomatic decreases in serum calcium and phosphate may occur, especially in patients receiving glucocorticoids, in whom calcium absorption may be decreased

Osteonecrosis of external auditory canal has been reported with bisphosphonates, mainly in association with long-term therapy. Possible risk factors for osteonecrosis of the external auditory canal include steroid use and chemotherapy and/or local risk factors such as infection or trauma. The possibility of osteonecrosis of the external auditory canal should be considered in patients receiving bisphosphonates who represent with ear symptoms including chronic ear infections.

Pediatric use: Apo-Alendronate in pediatric patients have not been studied, and do not administer this medicine them.

Geriatric use: In clinical studies aren't differences in efficacy or safety were observed between these patients and younger patients.

DRUG INTERACTIONS

If taken at the same time, it is likely that food and beverages (including mineral water), calcium supplements, antacids, and some oral medicinal products will interfere with absorption of alendronate. Therefore, patients must wait at least 30 minutes after taking alendronate before taking any other oral medicinal product. No other interactions with medicinal products of clinical significance are anticipated. No adverse experiences attributable to alendronate and oestrogen concomitant use were identified. Alendronate was used concomitantly with a wide range of commonly prescribed medicinal products without evidence of clinical adverse interactions.

USE IN PREGNANCY AND LACTATION

Apo-Alendronate has not been studied in pregnant women and breastfeeding women and should not be given to them.

ADVERSE REACTIONS

The following adverse reactions have been reported:

Body as a Whole: hypersensitivity reactions including urticaria and rarely angioedema. As with other bisphosphonates, transient symptoms as in an acute-phase response (myalgia, malaise, asthenia and rarely, fever) have been reported, typically in association with initiation of treatment. Rarely, symptomatic hypocalcemia has occurred, generally in association with predisposing conditions.

Osteonecrosis of the external auditory canal (bisphosphonate class adverse reaction), has been reported very rare.

Gastrointestinal: nausea, vomiting, esophagitis, esophageal erosions, esophageal ulcers, rarely esophageal stricture or perforation, and oropharyngeal ulceration; rarely, gastric or duodenal ulcers, some severe and with complications.

Localized osteonecrosis of the jaw, generally associated with tooth extraction and/or local infection, with delayed healing has been reported rarely.

Musculoskeletal: bone, joint, and/or muscle pain, rarely severe and/or incapacitating.

Nervous System: dizziness, vertigo.

Skin: rash (occasionally with photosensitivity), pruritus, rarely severe skin reactions, including Stevens-Johnson syndrome and toxic epidermal necrolysis.

Special Senses: rarely uveitis, scleritis or episcleritis.

SYMPTOMS AND TREATMENT OF OVERDOSAGE

No specific information is available on the treatment of overdose with Apo-Alendronate. Hypocalcemia, hypophosphatemia, and upper gastrointestinal adverse events; such as upset stomach, heartburn, esophagitis, gastritis, or ulcer, may result from oral overdose. Milk or antacid should be given to bind alendronate. Due to the risk of esophageal irritation, vomiting should not be induced and the patient should remains fully upright.

STORAGE CONDITIONS

Store below 30°C.

AVAILABILITY OF DOSAGE FORMS

APO-ALENDRONATE 70 mg: each white, oval, biconvex tablet, engraved “APO” on one side and “ALE70” on one side, contains alendronate sodium trihydrate equivalent to 70 mg alendronic acid 70mg. Available in Alu/PVC blister of 4's/box.

MANUFACTURER

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